

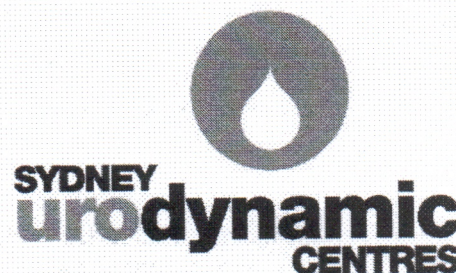
CB/dd

8th October 2025

Dr L Guan
Burwood Westfield Medical Centre
Shop 331, Level 2, 100 Burwood Road
BURWOOD NSW 2134

Dear Ling,

RE: Ms Yujie Cao DOB: 08/08/97
15/51-55 Shaftbury Road, Burwood 2134



ABN No: 53 866 387 150

PARTNERS

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Thank you for referring this pleasant 28 year old lady for urodynamic assessment. As you are aware, she has a long history of bladder concerns dating back to her childhood. Yujie is troubled by urinary frequency and urgency but also urge incontinence most days. There may be occasional stress incontinence. This does affect her quality of life. She has seen a Physiotherapist and had a trial of TENS treatment without great success.

Yujie is generally well and is nulliparous. There are no particular menstrual problems. She probably decreases her fluid intake which is a likely factor in her constipation.

On examination, Yujie was not overweight. No abnormality was found on abdominal palpation. On pelvic assessment, there was no significant prolapse and no masses or tenderness. She has a retroverted uterus.

UROFLOWMETRY: she had a low peak flow rate of only 12mls/sec with a somewhat intermittent voiding pattern. Her initial residual was 60mls but 300mls at the end of the study. This suggests possible voiding dysfunction. However, Yujie has had some bowel issues today with diarrhoea and this may have inhibited her voiding.

DYNAMIC URETHROSCOPY: no abnormality was seen in the urethra or bladder. There is no evidence of polyps, calculi or interstitial cystitis. She had a normal urethral closure pressure of 63cmsH₂O.

CYSTOMETROGRAM: she had her first sensation to void at 100mls and a bladder capacity of only 300mls. There were several episodes of abnormal **detrusor overactivity** which was associated with urinary leakage on one occasion.

IN CONCLUSION: Yujie has significant detrusor overactivity. No stress incontinence could be demonstrated today. There may be an element of voiding dysfunction and this needs to be monitored. I have discussed these findings with her. I have asked her to complete a bladder diary and to minimise caffeine intake. Bladder retraining and pelvic floor exercises are indicated. I have suggested a trial of Betmiga, 50 milligrams daily and given her a prescription for this. Her urinary residuals need to be monitored and I would be happy to do this if you and she would like.

With kind regards,